SCI MYS-001
R.O.C. Reg. No.:05481

SHINCORT Solution for Injection 2mg/ml

Ingredient(s)
Each ml contains:
Triamcinolone acetonide ……..……. 2 mg Benzyl alcohol (preservative) ……..……. 0.02 ml

Pharmacodynamics
Triamcinolone acetonide is a synthetic glucocorticoid and have effects on virtually every cell type and system in mammals.
Cardiovascular system:
Glucocorticoids can reduce capillary permeability and enhance vasoconstriction. A relatively clinically insignificant positive inotropic effect can occur after glucocorticoid administration. Increased blood pressure can result from both the drugs' vasoconstrictive properties and increased blood volume that may be produced.
Cells:
Glucocorticoids inhibit fibroblast proliferation, macrophage response to migration inhibiting factor, sensitization of lymphocytes and the cellular response to mediators of inflammation. Glucocorticoids stabilize lysosomal membranes.
CNS / Autonomic nervous system:
Glucocorticoids can lower seizure threshold, alter mood and behavior, diminish the response to pyrogens, stimulate appetite and maintain alpha rhythm. Glucocorticoids are necessary for normal adrenergic receptor sensitivity.
Endocrine system:
When animals are not stressed, glucocorticoids will suppress the release of ACTH from the anterior pituitary, thereby reducing or preventing the release of endogenous corticosteroids. Stress factors (e.g., renal disease, liver disease, diabetes) may sometimes nullify the suppressing aspects of exogenously administered steroids. Release of thyroid-stimulating hormone (TSH), follicle-stimulating hormone (FSH), prolactin and luteinizing hormone (LH) may all be reduced when glucocorticoids are administered at pharmacological doses. Conversion of thyroxine (T4) to triiodothyronine (T3) may be reduced by glucocorticoids; and plasma levels of parathyroid hormone increased. Glucocorticoids may inhibit osteoblast function. Vasopressin (ADH) activity is reduced at the renal tubules and diuresis may occur. Glucocorticoids inhibit insulin binding to insulin-receptors and the post-receptor effects of insulin.
Hematopoietic system:
Glucocorticoids can increase the numbers of circulating platelets, neutrophils and red blood cells, but platelets aggregation is inhibited. Decreased amounts of lymphocytes (peripheral), monocytes and eosinophils are seen as glucocorticoids can sequester these cells into the lungs and spleen and prompt decreased release from the bone marrow. Removal of old red blood cells is diminished. Glucocorticoids can cause involution of lymphoid tissue.
GI tract and hepatic system:
Glucocorticoids increase the secretion of gastric acid, pepsin and trypsin. They alter the structure of mucin and decrease mucosal cell proliferation. Iron salts and calcium absorption are decreased while fat absorption is increased. Hepatic changes can include increased fat and glycogen deposits within hepatocytes, increased serum levels of alanine aminotransferase (ALT) and gamma-glutamyl transpeptidase (GGT). Significant increases can be seen in serum alkaline phosphatase levels. Glucocorticoids can cause minor increases in BSP (bromo-sulfophthalein) retention time.
Immune system:
Glucocorticoids can decrease circulating levels of T-lymphocytes; inhibit lymphokines; inhibit neutrophil, macrophage, and monocyte migration; reduce production of interferon; inhibit phagocytosis and chemotaxis; antigen processing; and diminish intracellular killing. Specific acquired immunity is affected less than nonspecific immune responses. Glucocorticoids can also antagonize the complement cascade and mask the clinical signs of infection. Mast cells are decreased in number and histamine synthesis is suppressed. Many of these effects only occur at high or very high doses and there are species differences in response.
Metabolic effects:
Glucocorticoids stimulate gluconeogenesis. Lipogenesis is enhanced in certain areas of the body (e.g., abdomen) and adipose tissue can be redistributed away from the extremities to the trunk. Fatty acids are mobilized from tissues and their oxidation is increased. Plasma levels of triglycerides, cholesterol and glycerol are increased. Protein is mobilized from most areas of the body (not the liver).
Musculoskeletal:
Glucocorticoids may cause muscular weakness (also caused if there is a lack of glucocorticoids), atrophy, and osteoporosis. Bone growth can be inhibited via growth hormone and somatomedin inhibition, increased calcium excretion and inhibition of vitamin D activation. Resorption of bone can be enhanced. Fibrocartilage growth is also inhibited.
Ophthalmic:
Prolonged corticosteroid use (both systemic or topically to the eye) can cause increased intraocular pressure and glaucoma, cataracts and exophthalmos.
Renal, fluid, & electrolytes:
Glucocorticoids can increase potassium and calcium excretion; sodium and chloride reabsorption and extracellular fluid volume. Hypokalemia and/or hypocalcemia occur rarely. Diuresis may occur following glucocorticoid administration.
Skin:
Thinning of dermal tissue and skin atrophy can be seen with glucocorticoid therapy. Hair follicles can become distended and alopecia may occur.

Pharmacokinetics
Glucocorticoids are well absorbed after oral administration and have a bioavailability of 60~100%. In parenteral form, esterification of a glucocorticoid affects its water and lipid solubility and the rate at which is absorbed from the injection site. Acetate and acetonide esters are poorly water-soluble and result in slow and sustained absorption from intramuscular depot injections. Glucocorticoids have moderate protein binding and a moderate apparent volume of distribution. Because glucocorticoids act intracellularly, plasma half-life may not correlate well with the duration of biological effect. In dogs, the plasma half-life of triamcinolone is approximately 1.95 hours after IV administration.
Hepatic metabolism is the main elimination pathway of corticosteroids in animals; however, bililary excretion, renal metabolism, and renal clearance contribute to a small degree in some species. In addition, many glucocorticoids are administered in a form that must undergo hydrolysis or hepatic biotransformation to become active.
Glucocorticoid metabolism is a two-step process. Firstly, oxygen or hydrogen atoms are added then secondly, conjugation takes place (glucuronidation or sulphation). Subsequently the kidney excretes the resulting hydrophilic inactive metabolites. Intracellular metabolism by 11β-hydroxysteroid dehydrogenase (11β-HSD) controls the availability of glucocorticoids for binding to the glucocorticoid and mineralocorticoid receptors. Type 1 dehydrogenase (11β-HSD1) is widely distributed in glucocorticoid target tissues and has its highest activity in the liver. 11β-HSD1 acts mainly as a reductase, converting the inactive cortisone to the active cortisol.
In dogs, elimination of triamcinolone is equally divided between feces and urine after an IV dose of 2mg/kg. Only 20% of injected triamcinolone is recovered as unchanged drug in the urine, although 90% of the injected drug is recovered in 120 hours.

Indication
1. Treatment of inflammation and related disorders in dogs and cats.
2. Management and treatment of acute arthritis, allergic and dermatologic disorders in dogs and cats.

Target Species
Dogs and cats.

Recommended Dose
Intramuscular or subcutaneous:
Single injection of 0.1~0.2 mg (0.05~0.1 ml) per kg body weight.

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Remission of symptoms usually lasts 7~15 days. Dose may be repeated if symptoms recur after this time.
Intralesional:
1.2~1.8 mg (0.6~0.9 ml), inject circumscribed around the lesion in various sites.
Injections should be spaced 0.5~2.5 cm apart. Do not exceed 0.6mg (0.3 ml) at one injection site and do not exceed a total dose of 6 mg (3 ml).
Intraarticular and intrasynovial:
Single injection of 1~3 mg (0.5~1.5 ml).

Mode of Administration
By intramuscular, subcutaneous, intralesional, intraarticular and intrasynovial injection.

Contraindication
1. Systemic use of glucocorticoids is generally considered contraindicated in systemic fungal infections (unless used for replacement therapy in Addison's), when administered IM in animals with idiopathic thrombocytopenia and in animals hypersensitive to a particular compound.
2. Sustained-release injectable glucocorticoids use is considered contraindicated for chronic corticosteroid therapy of systemic diseases.

Warning and Precautions
1. Animals that have received glucocorticoids systemically, other than with 'burst' therapy, should be tapered off the drugs. Animals which have received the drugs chronically should be tapered off slowly as endogenous ACTH and corticosteroid function may return slowly. Should the animal undergo a 'stressor' (e.g., surgery, trauma, illness, etc.) during the tapering process or until normal adrenal and pituitary function resume, additional glucocorticoids should be administered.
2. Corticosteroid therapy may induce parturition in large animal species during the latter stages of pregnancy.
3. Mice, rabbits, and rats are considered steroid-sensitive because the administration of glucocorticoids causes a rapid lymphopenia induced by lymphocytolysis rather than the redistribution of circulating lymphocytes seen in other animal species, such as cattle, dogs, goats, horses, and pigs.

Interaction with Other Medicaments
1. Amphotericin B or potassium-depleting diuretics (furosemide, thiazides) when administered concomitantly with glucocorticoids may cause hypokalemia. When these drugs are used concurrently with digitalis glycosides, an increased chance of digitalis toxicity may occur should hypokalemia develop.
2. Glucocorticoids may reduce salicylate blood levels.
3. Insulin requirements may increase in animals taking glucocorticoids.
4. Phenytoin, phenobarbital, rifampin may increase the metabolism of glucocorticoids.
5. Concomitant administration of glucocorticoids and cyclosporine may increase the blood levels of each, by mutually inhibiting the hepatic metabolism of each other.
6. Glucocorticoids may also inhibit the hepatic metabolism of cyclophosphamide.
7. Mitotane may alter the metabolism of steroids; higher than usual doses of steroids may be necessary to treat mitotane-induced adrenal insufficiency.
8. Animals taking corticosteroids at immunosuppressive dosages should generally not receive live attenuated-virus vaccines as virus replication may be augmented. A diminished immune response may occur after vaccine, toxoid, or bacterin administration in animals receiving glucocorticoids.
9. Administration of ulcerogenic drug (e.g., non-steroidal anti-inflammatory drugs) with glucocorticoids may increase the risk of GI ulceration.
10. The effects of hydrocortisone, and possibly other glucocorticoids, may be potentiated by concomitant administration with estrogens.
11. In animals with myasthenia gravis, concomitant glucocorticoid and anti-cholinesterase agent (e.g., pyridostigmine, neostigmine, etc.) administration may lead to profound muscle weakness.

Pregnancy and Lactation
1. Glucocorticoids are probably necessary for normal fetal development. They may be required for adequate surfactant production, myelin, retinal, pancreas and mammary development.
2. Excessive dosages early in pregnancy may lead to teratogenic effect.
3. In horses and ruminants, exogenous steroid administration may induce parturition when administered in the latter stages of pregnancy.
4. Corticosteroid administered systemically during the last trimester of pregnancy may induce the first stage of parturition. If induced prematurely, labor may be followed by dystocia, fetal death, retained placenta, and/or metritis in all species.
5. Offspring of dogs administered corticosteroids during pregnancy have had congenital anomalies, such as cleft palate, deformed forelegs, phocomelia (absence of the proximal part of a limb), or anasarca (generalized, massive edema).
6. In rabbits and rodents, corticosteroids administered during pregnancy have caused cleft palate.
7. Glucocorticoids unbound to plasma proteins will enter milk. High dosages or prolonged administration to mothers may potentially inhibit the growth of nursing newborns.

Side Effects
1. Adverse effects are generally associated with long-term administration of these drugs, especially if given at high dosages or not on an alternate day regimen. Effects generally are manifested as symptoms of hyperadrenocorticism. When administered to young, growing animals, glucocorticoids can retard growth.
2. In dogs, polydipsia (PD), polyphagia (PP) and polyuria (PU), may all be seen with short-term 'burst' therapy as well as with alternate-day maintenance therapy on days when giving the drug. Adverse effects in dogs can include dull, dry haircoat, weight gain, panting, vomiting, diarrhea, elevated liver enzymes, pancreatitis, GI ulceration, lipidemias, activation or worsening of diabetes mellitus, muscle wasting and behavioral changes (depression, lethargy, viciousness). With the exception of PU/PD/PP, adverse effects associated with anti-inflammatory therapy are relatively uncommon. Adverse effects associated with immunosuppressive doses are more common and potentially more severe.
3. Cats generally required higher dosages than dogs for clinical effect, but tend to develop fewer adverse effects. Occasionally polydipsia, polyuria, polyphagia with weight gain, diarrhea, or depression can be seen. Long-term, high dose therapy can lead to 'Cushingoid' effects.
4. Administration of triamcinolone may play a role in the development of laminitis in horses.

Environmental Property
Not available.

Symptoms and Treatment of Overdose
1. An overdose tends to be defined as the point at which adverse effects outweigh beneficial effects rather than a specific mg per kg of body weight dose known to cause a toxic reaction. Glucocorticoids when given short-term are unlikely to cause harmful effects, even in massive dosages. One incidence of a dog developing acute CNS effects after accidental ingestion of glucocorticoids has been reported.
2. Treatment of overdose is discontinuation of therapy. Usually, a gradual withdrawal to avoid adrenal crisis is the primary treatment. When long-term treatment is essential to an animal, modification of dosing, such as alternate-day therapy, may be instituted to minimize adverse effects. Supportive treatment, such as medication to treat infection or GI disorders, also may be necessary.

Storage Condition
Store below temperature 30°C.

Shelf Life
3 years.
Use immediately after opening

To extend the shelf life from 2 years to 3 years

Withdrawal Periods
None.

Maximum Residual Limit (MRL)
None has been mentioned.

Product Description
A white suspension solution, and without other foreign matter.
Glass vial of 100 ml

Product of YSP
Manufacturer:
VETNOSTRUM ANIMAL HEALTH CO., LTD., HSINCHU PLANT
No. 290-3, Chung Lun, Chung Lun Village,
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Product Registration Holder:
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